



Impact & Benefits

The social, economic, governance, educational and user impact of getting every emergency patient to the right hospital the first time.

☆ MPOne Idea & Innovation Hackathon 2026 🏠 Challenge 5 · AI for Public Services · Healthcare

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1 The core impact: time to the right care

For heart attacks, strokes, severe trauma, burns and obstetric emergencies, minutes decide outcomes. Medreach removes the biggest avoidable delay: going to a hospital that cannot treat the patient and then travelling again.

- One journey instead of two: only hospitals that can treat the emergency *now* are offered.
- Hospital ready on arrival: the ER accepts, assigns a bay and reads the AI pre-arrival note before the patient arrives.
- No refusals at the gate: acceptance happens before travel; if one hospital declines, the next is asked automatically.
- Nobody left behind: "I'm alone" mode acts for people who cannot make calls themselves.

Illustrative scenario: chest pain at 11 pm

Today	With Medreach
Drive to the nearest hospital	Speak the emergency in Hindi
No cardiologist / ICU full	Only capable hospitals shown
Referred onward, second trip	Hospital accepts, bay Resus-02
ER unprepared on arrival	ER read the SBAR note in advance

Scenario for illustration; real time savings will be measured in the pilot (see KPIs below).

2 Impact across all five dimensions



Social impact

Fewer preventable deaths and disabilities from delayed care. Equal access for rural families, the elderly, people living alone and those who only speak Hindi.



Economic impact

Families avoid repeat transport and wasted hospital visits. Hospitals use ICU and specialist capacity better. Earlier treatment reduces long-term care costs and lost income.



Governance impact

A live, audited picture of emergency capacity across hospitals. Evidence for planning beds, specialists and ambulances. Transparent, explainable decisions.



Educational impact

Safe first-aid steps while help is arranged (e.g. CPR guidance), clear "this is not a diagnosis" messaging, and awareness of Ayushman / ABHA and 108.



User impact

Calm, simple, bilingual experience in panic: one button, voice input, clear status (reported → accepted → confirmed) and a tracking link for family.



Health-system impact

Less ER overcrowding from misdirected patients, fewer inter-hospital transfers, better use of government and Ayushman-empanelled hospitals.

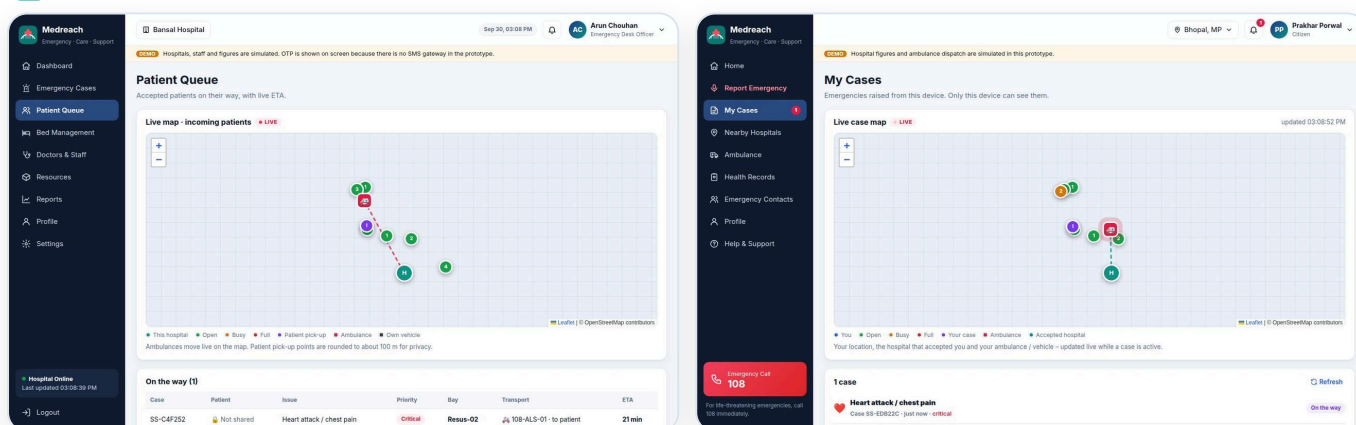
3 Benefits for every stakeholder

Stakeholder	Benefit
Patient	Treated sooner, by the right specialist, with the bed and bay already prepared.
Family / trusted contact	Knows the destination and ETA through a live link; no medical details are exposed.
Hospital ER team	Advance notice, severity, needs and SBAR note; can accept or decline with one tap; fewer unannounced critical arrivals.
Hospital management	Live bed / ICU / equipment board, arrival forecast for staffing, audit log of every action.
108 / ambulance service	Every trip goes to a destination that has already accepted the patient (integration planned via the 108 control room).
Govt. of MP / Health Dept.	State-wide view of emergency capacity and referral delays; data for policy and investment decisions.
ASHA / community health workers	Can raise an emergency for a villager by voice, in Hindi, from any phone browser.

4 How we will measure impact (pilot KPIs)

 Time to definitive care call → treating hospital	 Secondary transfers avoided patients who needed a second hospital	 Referral acceptance time request → hospital accepts
 Data freshness % of hospitals verified <15 min	 Refusals at the gate arrivals not accepted	 Citizen satisfaction short feedback after hand-over

5 Impact visible in the product today



Hospital sees incoming patients and ambulances live, with ETA

Citizen follows the case: accepted, ambulance on the way

Scalable impact: the same platform works for any district of Madhya Pradesh and any emergency type. Adding a hospital only needs its staff to log in and keep figures current, which takes one click to confirm.